

I consider moments where someone would say, *Your brain is not supposed to work like that*, and I ask: *Why?* I have two chief interests; the scientifically open question about how brains are shaped and the metaethics of well being saying it should be formed in a certain way.

In 2024, substance use disorder (SUD) was a top-ten leading cause of death worldwide. The claim: *You are not supposed to use opioids that way*. My role as a philosopher of science and values is to assess how philosophers and psychiatrists are making sense of these claims. What makes something a mental disorder? Whose values are we listening to and why? As it stands, there is disagreement about what SUD (addiction) is. So the first thing I do in my research is argue for a view of addiction that we should agree on, based on the current state of the literature.

The mind is my overarching focus: what it is, what makes it healthy, and how it is talked about. I am interested in questioning the diagnostic criteria of more individual diagnoses (e.g. dissociative identity disorder, gambling disorder). I will explain why, ethically and medically, we should not describe addiction as purely biological. Then, psychiatry aside, I will enter into discussions of personal identity and consciousness, emphasizing why the values used to inform our definitions of those phenomena matter. Many philosophers of mind debate about the constitution of the mind. *What is it made out of? Is it real?* I believe that answers to these questions should not be read as objective descriptions of nature, but as normatively-loaded arguments. I think that descriptions of the mind are communicating in terms of what makes sense when pursuing particular goals. But whose goals and why? Rather than arguing about the objective status of selfhood, or consciousness, I am interested in identifying which values justify these descriptions.

I have had the opportunity to present my research at the Philosophy of Science Association conference and participate in the competitive week-long Self-Control in Context workshop (hosted by instructors such as Chandra Sripada and Michael Inzlicht). While my dissertation has a connected topic throughout, such that reading one chapter helps to understand another, I also plan to publish all three chapters of my dissertation as standalone contributions within the next couple of years. While more details can be found in my dissertation abstract, below I explain my overall research program.

You reach for your phone again. You reach for a cigarette again. You reach for water again. How can you tell when your usage has gone too far? When you think about it every day? How much? When going without it brings you pain? How much pain? Whose standards are deciding these things? In psychiatry, a diagnosis of mental illness is often linked to what is called “clinically significant distress,” but that phrase is not itself standardized. Sometimes it references the patient’s idea of distress, and sometimes it references an external source’s idea of distress. When it comes to the diagnosis of addiction I argue that it is the patient’s idea of distress that matters.

The first contribution I make to the field is a clear explanation of patient-relative addiction diagnosis, where I contrast it with the understanding of addiction as “dependence.” It is a common misconception about addiction is that the individual is dependent. I provide a thorough explanation of why this description was a major source of disagreement among philosophers, and I offer an unintuitive description that is ultimately more representative of modern psychiatry. I argue that we ought to transition from an evaluatively negative interpretation of dependence to a strictly patient-relative teleological one, and, in doing so, SUD becomes best understood as persistent *unsuccessful* dependence. It is this understanding of addiction that leads us to focus less on dependence outright and more on how our desires are formed, how our values are formed, and how our desires compare to our values.

Since addictions are characterized by persistent unacceptable consequences, addiction is often considered *irrational*. A decision that leads to unacceptable consequences when preferable alternatives are available does not make sense. However, if that is the case, then why isn’t rational decision-making the remedy to, or even just a decent preventative measure against, addiction? *Just make decisions that have acceptable consequences*. Is that not the solution? I make clear that rational decision-making does not prevent addiction because addiction is a pathology of desire-formation, not of decision-making. In fact, engaging with addictive substances or behaviors, even before developing the addiction, is often already rational. My contribution in this discussion is to the field of decision-theory, showing that a simple game theoretic setup illustrates this critical psychiatric point.

It is critical to explain exactly why patient-values should trump something like an objective list theory of well-being. So I explain why, in addiction diagnosis, patient-values should be the goals referenced in the analysis of one’s dependencies (even if there truly was an objective list for well-being). To make this point, I connect feminist philosophical literature on adaptive preference to the ethics of psychiatric intervention. In the feminist literature on adaptive preference, there is a classic story of a fox who cannot reach some grapes. In the fox’s frustration, he convinces himself that the grapes are sour. So he stops wanting them. But does he really? With an adaptive preference, someone’s values have been influenced by factors outside their control, so there is an argument made that the desires are not their own. One could ask: can something be considered an addiction due to the fact that it is an adaptive preference? Suppose someone who, in dire circumstances, claims to successfully depend on alcohol to be happy. The current model of SUD says this person would not have SUD so long as her goals are genuinely achieved. However, what if her *ideal* goals were not being achieved? I explain why adaptive preferences understood this way are not addictions. This contributes to the field of addiction research by clarifying what kind of deleterious preferences actually contribute to pathological-use.

By the time we have verified that a patient is unsuccessfully dependent, that they suffer from irrational desires rather than irrational decisions, and that it is not just an adaptive preference, then the next logical step would be to try to help them. Western medicine has grown especially fond of finding biological interventions: pills, oils, injections, surgeries. What should we give people who are suffering from addiction? There is an old debate in the philosophy of psychiatry about whether mental disorders are purely biological; a purely biological illness has purely biological solutions. However, there is a gap in the *linguistics* literature about *what it should mean* for, say, addiction to be purely biological. The connection to linguistics here lives in the ethics behind the label. Philosophers have often asked: *In objective reality, are mental disorders purely biological?* That debate presumes that there is a fact of the matter. However, I do not think there is, and the answer changes the treatment we choose. I will explain why, linguistically, we shouldn't refer to addiction as strictly biological, even if there were a pinpointable biological mechanism driving addictive behavior. This contributes to the philosophy of language, showing how metalinguistic debates affect medical practice in the case of addiction diagnosis and treatment.

I also have an op-ed planned. The bulk of my research has been theory development and its clarification through responding to counterexamples. While work on addiction is meaningful to many people in the world, my work has been geared toward academics. So I am developing a public-facing piece that uses my work on addiction to argue against the promotional campaigns of popular gambling platforms like Kalshi—the federally regulated financial exchange in the United States where users bet on future outcomes. Gambling disorder is one of the most recent additions to the list of addictive disorders in the Diagnostic and Statistical Manual of Mental Disorders, and (put simply) it is characterized by the unwarranted prediction of future reward. Kalshi, and platforms like it, have made gambling far too easy and encouraging. Yet Kalshi attorneys will defend the platform by arguing that adults can spend their money how they want, even if it costs them their livelihood. I will explain why, contrary to the promoted defenses, the platform may be fostering gambling disorders and is thus perhaps not as safe as it may seem.